

Doctronic has emerged as a leader in AI-native clinical care, demonstrating that artificial intelligence can safely expand access while preserving physician oversight and accountability. In 2025, Doctronic's platform supported more than 20 million patient visits, reflecting scaled deployment of AI-powered triage, structured diagnostic reasoning, medication safety screening, and automated documentation workflows.

A defining milestone was Doctronic's work in Utah, which marked the first deployment of an autonomous AI system to process prescription refills within defined clinical guardrails. The system safely evaluated refill eligibility, screened for contraindications and drug interactions, and escalated exceptions to licensed physicians when risk thresholds were met. This initiative demonstrated that carefully bounded autonomous AI can manage high-volume, protocol-driven tasks while maintaining safety and compliance.

Importantly, Doctronic operates under a human-in-the-loop model for broader clinical care. The organization conducts approximately 15,000 human doctor-led encounters per month, with physicians reviewing AI-generated workups, validating care plans, and managing complex or high-risk cases.

The combination of scaled automation and physician oversight has enabled measurable expansion of access, reduced administrative burden, and improved operational efficiency—positioning Doctronic at the forefront of responsible, large-scale AI deployment in clinical care.

The following are key public comments which outlines our leadership in rolling out a transformative, native AI approach within the delivery of care model:

- Doctronic Study: AI Accuracy: <https://www.medrxiv.org/content/10.1101/2025.07.14.25331406v1>
- Foxnews: <https://www.foxnews.com/video/6376073821112>
- Fox Business: <https://www.youtube.com/watch?v=h5aJS7DBeXs&t=336s>
- CNBC Squawkbox: <https://www.cnbc.com/video/2026/01/16/future-of-ai-doctors-doctronic-co-founders-on-the-rise-of-ai-prescriptions.html>

HHS Request for Information: Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care

In addition to the general requests for information above regarding AI regulation, reimbursement, and research & development, HHS seeks input on the following specific questions:

1. What are the biggest barriers to private sector innovation in AI for health care and its adoption and use in clinical care?

a. Payment Misalignment

- Fee-for-service (FFS) rewards volume, not automation or prevention.
 - AI that reduces visits, admissions, or documentation time is often financially disincentivized.
- b. Regulatory Uncertainty
- Ambiguity between FDA-regulated medical devices and CDS exempt under 21st Century Cures Act § 520(o)(1)(E).
 - Lack of clear guidance for adaptive, continuous learning systems.
- c. Liability Ambiguity
- Unclear standards of care when AI assists in diagnostic reasoning.
- d. Interoperability Gaps
- Variable FHIR implementation despite 45 CFR Part 170 (ONC Health IT Certification Program).
 - EHR vendor friction and inconsistent API performance.
- e. Data Access & Quality
- Fragmented longitudinal records.
 - Limited structured data for benchmarking and validation.
- f. Trust & Governance
- Clinician skepticism without transparent evaluation standards.
 - Lack of standardized AI assurance frameworks.
- 2. What regulatory, payment policy, or programmatic design changes should HHS prioritize to incentivize the effective use of AI in clinical care and why?**

HHS should prioritize reforms that align incentives with prevention, automation, and measurable quality improvement—rather than visit volume.

- Modernize Payment Policy to Reward AI-Enabled Care

A. Recognize AI-Assisted Clinical Work Under the Physician Fee Schedule

Under 42 CFR Part 414, CMS should:

- Create reimbursement pathways for AI-assisted documentation, triage, and longitudinal monitoring.
- Clarify that time saved through automation does not reduce eligibility for E/M coding when clinical complexity remains.
- Recognize asynchronous, AI-augmented encounters as reimbursable services.

Why: AI increases efficiency and expands capacity, but current FFS rules penalize reductions in time or visit volume.

B. Integrate AI Performance into Value-Based Payment Models

Within:

- 42 CFR Part 425 (MSSP)
- ACO REACH (42 CFR Part 512)

CMS should:

- Explicitly recognize validated AI tools as quality and cost-reduction enablers.
- Allow shared savings credit for AI-driven reductions in ED visits, readmissions, and avoidable utilization.
- Encourage AI-enabled care redesign within primary care transformation models.

Why: AI's strongest ROI appears in risk-bearing environments. Value-based models naturally align with prevention and automation.

What HHS regulations, policies, or programs could be revisited to augment your ability to develop or use AI in clinical care? Please provide specific changes and applicable Code of Federal Regulations citations.

- Modernize Payment Models to Reimburse AI-Enabled Care

Key Regulations: 42 CFR Part 414 (Physician Fee Schedule), 42 CFR Part 425 (MSSP)

Change:

- Create reimbursement pathways for AI-supported triage, documentation, with
- in both synchronous and asynchronous care.
- Explicitly recognize AI-enabled efficiency gains in value-based care benchmarks.

Why: Current fee-for-service models reward visits, not prevention or automation.

- Clarify AI Clinical Decision Support Regulation

Key Authority: FD&C Act §520(o) (21st Century Cures Act CDS Exemption)

Change:

- Issue guidance for LLMs and adaptive AI systems.
 - Establish risk-tiered oversight based on clinical impact.
- Why: Regulatory ambiguity is one of the largest barriers to investment and deployment.

- Update HIPAA for AI Use Cases

Key Regulations: 45 CFR Parts 160 & 164

Change:

- Clarify PHI use for AI inference vs model training.
- Establish audit logging and AI decision traceability standards.
Why: Reduces compliance uncertainty while preserving privacy protections.

- Strengthen Interoperability Standards

Key Regulations: 45 CFR Part 170 (ONC Certification)

Change:

- Require consistent FHIR R4 implementation.
- Expand USCDI data elements for longitudinal and clinical reasoning data.
Why: AI performance depends on high-quality, standardized clinical data.

- Create AI Safety & Evaluation Demonstration Programs

Key Programs: CMS Innovation Center (42 USC §1315a)

Change:

- Fund real-world AI safety, bias, and effectiveness testing pilots.
Why: Accelerates evidence generation and clinician trust.

3. For non-medical devices, we understand that use of AI in clinical care may raise novel legal and implementation issues that challenge existing governance and accountability structures (e.g., relating to liability, indemnification, privacy, and security). What novel legal and implementation issues exist and what role, if any, should HHS play to help address them?

- Elected not to respond to #3

4. For non-medical devices, what are the most promising AI evaluation methods (pre- and post-deployment), metrics, robustness testing, and other workflow and human-centered evaluation methods for clinical care?

- a. Should HHS further support these processes? If so, which mechanisms would be most impactful (e.g., contracts, grants, cooperative agreements, and/or prize competitions)?**

- Elected not to respond to #4

5. How can HHS best support private sector activities (e.g., accreditation, certification, industry-driven testing, and credentialing) to promote innovative and effective AI use in clinical care?

HHS can accelerate AI adoption by promoting trusted innovation ecosystems, standardized testing, and voluntary certification programs.

- **Physician Fee Schedule**
AI can perform a range of complex cases, given safety guardrails, enhance policy to allow AI to handle clinical intake within the scope of an E&M visit while allowing the human provider to make the final clinical decision. The ultimate measure is outcomes. Our IA services suggest that you can maintain the current fee schedule while reducing the cost of care over time.
- **Transparency & Trust Labels**
Create voluntary AI disclosure standards describing training methods, data provenance, and clinical validation results to support procurement decisions.
- **Innovation Demonstrations**
Expand CMS Innovation Center pilots (42 USC §1315a) to test AI-enabled care models tied to measurable outcomes and cost reduction.
- In many ways, CMS should let the market drive adoption and incentivize adapters to generate outcomes-based case studies inclusive of measures capturing safety, access, outcomes and costs.

Goal: Enable safe, scalable AI innovation while maintaining clinical safety, privacy, and trust.

6. Where have AI tools deployed in clinical care met or exceeded performance and cost expectations and where have they fallen short?

a. What kinds of novel AI tools would have the greatest potential to improve health care outcomes, give new insights on quality, and help reduce costs?

The AI tools with the greatest potential to improve outcomes, generate new quality insights, and reduce costs are those embedded directly into care delivery and population health workflows. AI-native primary care platforms that provide structured triage, diagnostic support, and automated documentation under clinician supervision can expand access, reduce emergency department utilization, and alleviate clinician burnout. Longitudinal risk detection models that continuously analyze clinical, claims, and social

determinants data can identify rising-risk patients earlier and trigger preventive interventions that reduce hospitalizations and total cost of care. Medication optimization engines can decrease adverse drug events and polypharmacy risks. AI-driven post-discharge and chronic disease monitoring tools can reduce readmissions and improve control of high-cost conditions. Finally, quality intelligence systems that synthesize claims and clinical data to identify practice variation and care gaps can provide actionable insights for value-based performance improvement. The highest-impact AI tools are those that measurably improve prevention, efficiency, and risk management rather than automate isolated administrative tasks.

7. Which role(s), decision maker(s), or governing bodies within health care organizations have the most influence on the adoption of AI for clinical care?

AI adoption in clinical care is driven by enterprise leadership focused on cost, quality, risk, and workforce impact—not solely by IT.

- Health Systems
 - Executive Leadership (CEO & Board): Sets strategic transformation priorities.
 - Clinical Leadership (CMO & Quality Committees): Validates safety, standard-of-care integration, and physician adoption.
 - Finance Leadership (CFO & Finance Committee): Evaluates ROI under FFS vs. value-based contracts.
 - Board / Risk Committees: Oversee liability, compliance, and patient safety.

Adoption typically requires clinical and financial alignment.

- Commercial & Medicare Advantage Payers
 - Executive Leadership Team – Sets cost containment and innovation strategy.
 - Medical Management Leadership – Assesses impact on utilization, Stars ratings, and MLR.
 - Actuarial & Finance Committees – Model risk adjustment and savings.
 - Population Health Leadership – Drives prevention and care management AI use.
- Medicare (CMS & CMMI)
 - CMS Leadership & Innovation Center (CMMI) – Determines payment model alignment and demonstration pilots.
 - Policy & Actuarial Offices – Evaluate regulatory compliance and budget neutrality.

- o AI adoption is left to the individual health system/practice; It's typical for their actions to be heavily influenced by their EHR vendor. An effort often reinforced with pricing policies to compel use of their modules before using a 3rd party.
- State Medicaid Programs
 - o Medicaid Director & State Health Agencies – Approve AI-enabled program adoption.
 - o This is a segment where dollars need to be stretched and AI can greatly influence increased access and outcomes at a reduced cost; however, contract agencies often create significant obstacles to adoption.
- Self-Insured Employers
 - o Benefits Leadership / CHRO – Drive access, workforce productivity, and cost strategy.
 - o Finance Leadership – Evaluate healthcare spend reduction and ROI.
 - o Third-Party Administrators (TPAs) – Influence implementation within benefit structures.

Across all segments, AI adoption hinges on demonstrated cost reduction, quality improvement, and risk mitigation.

a. What are the primary administrative hurdles to the adoption of AI in clinical care?

8. Where would enhanced interoperability widen market opportunities, fuel research, and accelerate the development of AI for clinical care? Please consider specific data types, data standards, and benchmarking tools.

- Clinical data remains siloed across different systems: medications (Surescripts), EHRs, CareQuality, Commonwell, EPIC, EPIC/QHIN. CommonWell QHIN, State HIEs, etc... Even many QHINs continue to hedge their bets by simultaneously supporting TEFCA/QHIN while also supporting their own networks – even adoption today into a private network further delays the realization of what TEFCA/QHIN can deliver. It is recommended HHS mandate TEFCA/QHIN interoperability within the next 12 months – minimally not allow entities to offer both a QHIN and their private network.

9. What challenges within health care do patients and caregivers wish to see addressed by the adoption and use of AI in clinical care? Equally, what concerns do patients and caregivers have related to the adoption and use of AI in clinical care?

- The inhibitor to AI adoption is largely due to the risk adverse models of health systems and payers. We have performed over 20M AI first clinical encounters in 2025 through Direct-to-Consumer programs. While there is work to do to enhance the experience the model continues to expand. Health systems are mixed some believing demand has outgrown clinician capacity, driving long wait times, rising costs, and inconsistent preventive and chronic-care management; hence believing AI is an approach to close the gap and facilitate scaling of their services. Other remain in the sideline.

10. Are there specific areas of AI research that HHS should prioritize to accelerate the adoption of AI as part of clinical care?

- a. Are there published findings about the impact of adopted AI tools and their use clinical care?**
 - b. How does the literature approach the costs, benefits, and transfers**
- Elected not to respond #10